FACILITATOR EDITION | KEEP PRIVATE UNTIL THE DEBRIEF

Instructor & Moderator Guide

Five-stage progressive-disclosure exercise · Case 001: The Vaccine Trial Nobody Fully Understood

THE CASE IN ONE SENTENCE

Can research designed to protect infants from a dangerous outbreak still violate research ethics when its consent process is incomplete?

Session specifications

Item	Recommendation
Audience	Upper-level undergraduates, MPH/DrPH students, health professionals, IRB/community advisory trainees
Group size	4–6 students per table; one facilitator and one scribe per group
Timing	70–80 minutes standard; 50 minutes compressed
Materials	Five-stage web app or vignette cards; the fictional consent form; decision log; full fact sheet held until debrief
Outcome	An ethically justified decision at each stage, revised as new evidence emerges

Learning objectives

- Apply autonomy, beneficence, non-maleficence, and justice to population-level research decisions.
- Distinguish parental permission, research informed consent, and infant assent (not generally possible for infants).
- Differentiate unlicensed/investigational status, vaccine dose, safety signal, causal findings, and U.S.-licensed routine vaccination.
- Identify roles of investigators, IRBs, sponsors, health departments, and community participants.
- Explain how transparency and accountability affect trust independent of observed injury.

How to stage the materials without spoiling the exercise

At the start, distribute only Stage 1 and the measles basics from page 1 of the fact sheet (or read those facts aloud). DO NOT distribute page 2 of the fact sheet: it identifies the investigational EZ vaccine and later safety findings. Hand out the fictional consent form at Stage 2. Reveal the rest sequentially. Distribute the complete fact sheet and sources only at Stage 5.

GROUND RULES

This is research-ethics analysis, not a debate about whether routine measles vaccination works. Avoid assuming intent, assigning guilt beyond documented findings, or implying that a signal from high-titer EZ studies proves a problem with routine MMR.

Instructor preparation

Review the primary 1997 congressional testimony (David Satcher, pp. 21–23) and current HHS informed-consent guidance. If students are using the web app individually, ask them to remain at each stage until the instructor releases the next stage; this static prototype does not lock student screens or aggregate responses.

RUN OF SHOW

Minute-by-minute plan

A structured format that leaves room for disagreement

Minutes	Facilitator action	Student output
0–5	Set ground rules; brief measles context without trial details.	Initial assumptions.
5–15	Reveal Stage 1. Give groups roles as a public health advisory committee.	Approve / revise / reject + conditions.
15–28	Reveal Stage 2; hand out simulated consent. Ask for markup.	Specific wording changes; consent determination.
28–40	Reveal Stage 3: external mortality safety signal.	Pause/stop/continue decision; communication plan.
40–52	Reveal Stage 4: consent deficiency becomes public.	Accountability and community-engagement response.
52–64	Reveal Stage 5: historical record and follow-up.	Revised conclusions and institutional reforms.
64–80	Facilitated plenary and closing reflection.	Defensible ethical judgment + one open question.

Reusable question after every reveal

ASK THE GROUP

What would you decide with ONLY the information available now? Which new fact changed your decision, and is that change based on evidence, an ethical obligation, or emotion?

Decision log template

Stage	Our decision	New information that matters	Principle in tension
1	_____	_____	_____
2	_____	_____	_____
3	_____	_____	_____
4	_____	_____	_____
5	_____	_____	_____

How to moderate

- Ask for a decision before inviting explanations. Avoid rewarding certainty; reward explicit assumptions and thresholds for changing course.
- Invite a counterargument: “What would a reasonable parent, investigator, or community organizer say in response?”
- Do not allow hindsight to erase uncertainty present at earlier stages; do not allow uncertainty to excuse incomplete disclosure.
- Use neutral language: say “a safety signal was observed,” not “the vaccine caused the Los Angeles deaths.”

REVEALS 1–3 | FACILITATOR NOTES

Stage cards & probes

Release only one at a time; preserve uncertainty

Stage 1 · The outbreak and the opportunity (5–15 min)

Reveal: During a serious measles epidemic, infants under the usual vaccine age are experiencing preventable disease. A study of earlier vaccination using EZ-based schedules is proposed.

Decision prompt: Under what conditions, if any, would the advisory group allow an infant vaccine study?

Probe: What benefits might accrue to infants, future cohorts, and the community? What alternative strategies could reduce measles exposure while research proceeds?

Look for: scientifically valid question, community relevance, equitable enrollment, independent review, feasible routine-care alternatives, safety monitoring, and accessible consent. Do not imply an outbreak suspends research protections.

Stage 2 · The parental permission document (15–28 min)

Reveal: Parents are asked to participate. Give students the simulated consent form and ask them to review it AS WRITTEN before discussing the historical findings.

Decision prompt: Would you approve this document as an IRB reviewer? If not, what exact additions are required?

Probe: Is “have been used to prevent measles in children” a sufficient description of the study products? Does mentioning “usual care” adequately describe alternatives? Is it enough that parents signed?

Look for: explicit research status of each arm; which product is not U.S.-licensed; investigational dose/age; specific foreseeable risks and unknowns; alternatives including nonparticipation; follow-up and contact details; how questions and understanding are handled.

Stage 3 · The safety signal (28–40 min)

Reveal: Evidence from studies in Senegal and Haiti raises concern about lower long-term survival among girls receiving high-potency measles vaccine. Mechanism and generalizability remain uncertain; the same harm has not been documented locally.

Decision prompt: Continue, pause selected arms, suspend all study vaccinations, or terminate? Who must be notified and when?

Probe: What if the early antibody response is excellent? What is the risk of waiting for a more definitive causal explanation?

Look for: immediate independent data review; clear stopping rules; participant notification; attention to sex-specific risks and longer follow-up; rigorous comparison groups; proportional protective action. In the actual Los Angeles case, EZ use was stopped in October 1991. [1]

REVEALS 4–5 | FACILITATOR NOTES

Disclosure, accountability & final record

The dilemma is not resolved by a good intention or a good outcome

Stage 4 · The consent problem becomes public (40–52 min)

Reveal: Parents participated in a research study but were not clearly told one of the vaccines was experimental/unlicensed in the United States. Most participants were from Black and Latino communities. Reviewers now question the adequacy of consent and the fairness of recruiting underrepresented communities.

Decision prompt: How should the institution respond to families and the public? Who leads this response?

Probe: What can be said about the basis for site selection, and what cannot be inferred about intent? What would meaningful community oversight look like?

Look for: candid disclosure, accessible family notification, independent investigation, publication of findings, support for follow-up, language access, community governance, and an apology that identifies what was wrong rather than merely expressing regret.

Stage 5 · Historical reveal (52–64 min)

Documented facts: The CDC supported a randomized, five-group study of about 1,500 infants beginning in June 1990. The CDC and Kaiser IRBs approved the protocol. In October 1991, CDC stopped EZ administration after external high-titer safety signals. Follow-up found no significant vaccine-attributable health problem among the Los Angeles high-potency recipients. A 1995 federal review described the study as scientifically and ethically justified but found that consent was deficient, especially around investigational status, foreseeable risks, and alternatives. Families received notice and an institutional apology in 1996. [1]

Decision prompt: If you were asked to evaluate the study in 1995, would you describe it as ethical, unethical, or mixed? Require students to explain what they are evaluating: research question, design, conduct, consent, safety oversight, or response.

Probe: Should a consent violation count as serious even if no injury attributable to the studied product is demonstrated? Would the response differ if the vaccine had been very effective?

KEY HISTORICAL NUANCE

Families were NOT all unaware that a study was taking place. Consent was signed, but material information was inadequately disclosed. The foreign safety signals should NOT be transposed into claims of U.S. MMR harms or vaccine-attributable deaths in this Los Angeles cohort.

Close with a practical choice

Ask each group to recommend: (1) an action in the next 24 hours, (2) one oversight change within three months, and (3) one measure of improved informed consent or community trust within a year.

ANSWER KEY | USE AFTER DISCUSSION

Consent-form audit

Why this fictional form would not be adequate

Phrase / omission	Why it matters	Acceptable revision
"The vaccines being compared have been used…"	True in a broad global sense but masks different U.S. licensure and investigational status.	Identify Moraten as U.S.-licensed and EZ as unlicensed/investigational in U.S.; specify which groups can receive which.
"At different strengths and ages"	Does not explain the particular experimental dose or age-related uncertainty.	Explain dose strengths and ages plainly; state what is known and unknown for each assignment.
"May help protect your child"	Potential benefit is plausible but not certain, especially at early age.	Separate individual benefit from benefits to scientific knowledge; give realistic expectations.
"Some children may have fever…"	Lists common reactions but not a substantive discussion of foreseeable risk/uncertainty.	Disclose reasonably foreseeable risks as known at the time; acknowledge gaps, monitoring and how new risks will be shared.
"Usual medical care"	Does not identify concrete alternatives or vaccination options.	Explain routine standard vaccine timing, outbreak options, and care if declining research.
No independent contacts or injury plan	Makes it harder to ask questions, report injury, or contact the IRB.	Provide study/IRB contacts and applicable injury-care information.
Signature at the end	A signature documents a decision; it does not prove understanding or adequate disclosure.	Include a comprehensible dialogue, teach-back, language access, time for questions, and voluntary choice.

Four guiding principles (discussion rather than verdict)

Autonomy / respect for persons: Parental permission requires meaningful disclosure; infants cannot meaningfully assent.

Beneficence: The outbreak and early-infant protection posed a compelling scientific and public health problem.

Non-maleficence: Monitoring and response to possible delayed harms mattered even before causality was settled.

Justice: Enrollment should correspond to disease burden without unfairly shifting research burdens, withholding alternatives, or excluding communities from oversight.

Avoid these common discussion errors

- Mistaking the EZ/high-titer investigational approach for the U.S.-licensed MMR vaccine.
- Claiming no one knew they were in research; the known deficiency concerned the scope and quality of disclosure.
- Assuming high-titer vaccine caused a death in the Los Angeles participants.
- Treating IRB approval or positive intentions as substitutes for effective ongoing consent.

ASSESSMENT & SOURCES

Debrief, assessment, and references

A strong conclusion makes room for evidence and justified disagreement

Suggested plenary prompts

- At which stage did your decision change most? What information drove that shift?
- Could the study have been ethically acceptable with better disclosure and safeguards?
- What is the difference between “not shown to have harmed” and “known to be harmless”?
- How should community members participate when researchers recruit in communities disproportionately affected by an outbreak?
- If you were the public health director, what would you want the families to learn from you first?

Optional reflection (300–500 words)

Identify a stage at which your group changed—or refused to change—its decision. Explain the strongest evidence available at the time, the competing ethical principles, the limits of your knowledge, and one institutional safeguard that could improve future decisions.

Suggested assessment rubric (10 points)

Criterion	Points
Clearly states an ethically defensible decision and the stage at which it applies.	2
Accurately distinguishes empirical evidence from uncertainty and hindsight.	3
Applies at least two relevant ethical principles in tension.	2
Proposes practical safeguards, including appropriate consent/accountability.	2
Treats communities and stakeholders fairly; acknowledges counterarguments.	1

Historical and ethics references

[1] D. Satcher, CDC testimony, U.S. House (1997), pp. 21–23. [Open source]
[2] HHS OHRP, Informed Consent FAQs. [Open source]
[3] HHS OHRP, 45 CFR 46, Subpart D: Protections for Children. [Open source]
[4] CDC, The Pink Book, Chapter 13: Measles. [Open source]
[5] WHO, “Safety of high titre measles vaccines,” 1992. [Open source]
[6] E.A. Holt et al., Differential Mortality by Measles Vaccine Titer and Sex, JID 1993. [Open source]
[7] CDC, Measles Vaccine Recommendations (2026). [Open source]
[8] Markowitz et al., infant vaccine dose trials, NEJM 1990 (research context). [Open source]

TEACHING INTEGRITY STATEMENT

The case uses documented historical events, but staged dialogue and the supplied consent form are fictional educational reconstructions. Historical statements should be traced to the sources above. Modern regulations help teach principles; do not automatically project every 2026 procedural requirement onto a 1990 trial.